



823 GATEWAY CENTER WAY
San Diego, CA 92102
Phone (619) 515-2368
Fax (619) 798-3697

Family Health Centers of San Diego

To:
Fax: 18587779551
Phone:

From: :W1:U1649:REST API
Company: User:Mailbox_Server_Workflow:R1
Fax: 1234567890
Phone:

Date: February 3, 2026
Total Pages: 25

Subject: Packaged Referral Documents

Comments:

Packaged Referral Documents

HIM QC and Data Integrity
Health Information Management Department
Main Number: (619) 515-2368
HIM QC Fax #: (619) 798-3697

Please deliver this FAX to the above-named person as quickly as possible.
If you have experienced any problems in receiving this communication, please call HIM at (619) 515-2368.

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HIM FAX COVER SHEET



Family Health Centers of San Diego Evaluation/Treatment Prescription

PACE COLLEGE GROVE

3420 COLLEGE AVE SAN DIEGO CA 92115-7134

Phone: (619)515-2445 Fax: (619)269-0707

Patient Identification

Name: **VASQUEZ, LUCIA**
 Date of Birth: **09/10/1953**
 Insurance: **PACE Family Health Center**
 Policy #: **101386778**
 Address: **4041 44TH ST**
 City, Zip: **SAN DIEGO 92105-1603**
 Phone No: **(619)964-6523**

You have been referred to:

OrderId: 116540498
 Specialty: HOME CARE (PACE ONLY)
 Facility: CARE, BRIGHTSTAR
 Address: 680 FLETCHER PARKWAY -201 EL CAJON 92020

Phone No: (858)777-9525

Please call (858)777-9525 to schedule or change your appointment.

Appt Date: _____ Appt Time: _____

Referred by: DAWUD, RENDA

Diagnosis: Z9181 At risk for falls

Reason for referral: 1x Order for Escort 02/04/2026 12:30pm-2:30pm1.CNA, Spanish Speaking, to serve as escort for medical appt2.Please meet participant at 12:30pm at 12:45pm at Lab4520 Executive Dr. Ste T2 San Diego, CA 920373.Assist as needed and remain with participant until safely picked up by PACE transportation

Service Type: ☐ Routine ☒ STAT

You are being referred for:

G0156(1) SERVICES OF HOME HEALTH/HOSPICE AIDE IN HOME HEALTH OR HOSPICE SETTINGS, EACH 15 MINUTES

Electronically Prescribed By
 Referring Provider

Date: 02/03/2026

(DAWUD, RENDA License: C177173 NPI: 1033131941)

Disclaimer: It is the specialist's responsibility to verify patient's insurance eligibility prior to delivering services. FHCS D will obtain and submit insurance authorization to the specialist's office for the initial consult, when necessary, but the specialist must again verify eligibility at the time of service. FHCS D is not responsible for any charges related to lack of coverage by patient's insurance company for services rendered by specialist.

PACE - Treatment Auth Form

FAMILY HEALTH CENTERS
OF SAN DIEGO

FamilyHealth – Center for Older Adults/PACE

3420 College Avenue, San Diego, CA 92115

P: (619) 515-2445 F: (619) 269-0707 www.fhcsd.org/PACE

TREATMENT AUTHORIZATION FORM

Please contact PACE for ALL appointment scheduling at 619-515-2445

Auth #:

27883588

Date Approved:

02/03/2026

Date Auth. Expire:

02/05/2026

UM Decision Status:

☒ APPROVED ☐ MODIFIED ☐ DEFERRED ☐ DENIAL

Comments:

1x Order for Escort 02/04/2026 12:30pm-2:30pm 1. CNA, Spanish Speaking, to serve as escort for medical appt 2. Please meet participant at 12:30pm at 12:45pm at Lab 4520 Executive Dr. Ste T2 San Diego, CA 92037 3. Assist as needed and remain with participant until safely picked up by PACE transportation

Reviewer's Name:

Juan Franco

Date:

02/03/2026

MEMBER (Participant)

Participant's Name:

VASQUEZ, LUCIA

DOB:

09/10/1953

Medical Record #:

1364779

Gender:

Female

Participant ID:

101386778

Participant's Current Address:

4041 44Th St San Diego, California 921051603

Primary Language Spoken:

SPANISH

Require Interpreter:

Yes

Referral/Service Type Requested

Service Type:

☐ ROUTINE ☐ URGENT ☐ RETROACTIVE

INPATIENT:

☐ Surgical Procedure
☐ SNF - Skilled
☐ SNF - Custodial
☐ ER Admission
☐ LTAC
☐ Hospital
☐ ALF

OUTPATIENT:

☐ Surgical Procedure
☐ Diagnostic
☐ Home Health
☐ Dialysis
☐ Specialty
☐ Other:

Home Care

INTERNAL:

☐ Home Care
☐ OT
☐ PT
☐ ST
☐ DME
☐ RN

☐ Initial Consult☐ F/U Visit(s):

Diagnosis Code and Description:

V1588 [I10: Z9181] - At risk for falls

CPT/HCPC Code and Description:G0156 - SERVICES OF HOME HEALTH/HOSPICE AIDE IN HOME HEALTH OR HOSPICE SETTINGS, EACH 15 MINUTES No: 1,
Auth: 1**Number of Visits Requested:****DOS from:**

02/04/26

Clinical Indications For Request:**Services to be provided at:****PROVIDER INFORMATION****Requesting Provider's Name:**

DAWUD, RENDA

Contact at Provider's Office:

3420 COLLEGE AVE SAN DIEGO CA 92115-7134

Fax:

(619) 269-0053

Facility/Provider Providing Service to:**Phone Number:**

(858)777-9525

Fax:

(858)777-9551

E-mail:**THIS REFERRAL DOES NOT GUARANTEE ELIGIBILITY. CHECK ELIGIBILITY PRIOR TO RENDERING SERVICE.****Payment will NOT be made for unauthorized services. All lab and x-rays must be ordered/performed by contracting providers (contact Family Health Centers PACE U.M. Department, Ph#: 619-515-2445, Fax#: 619-269-0707). Specialist reports must be sent to PCP promptly.****BILLING/CLAIMS INFORMATION:**Family Health Centers of San Diego, Inc.,
C/o Innermark
PO Box 21002
Eagan, MN 55121

For provider questions - 618-619-2184

For provider portal access - provideraccess@innermark.com**Electronic:** Institutional/Professional Payer ID: 98481

PACE - Authorization Request Form



3420 College Ave.
San Diego, CA 92115
Phone: (619) 515-2445
Fax: 619-269-0707



2201 Mission Ave., #200
Oceanside, CA 92058
Phone: (760) 826-7223
Fax: 619-269-0314

Authorization Request Form

PATIENT DIAGNOSIS

1.	2.	3.	4.
5.			

Patient MRN:

1364779		
Last Name:	First Name:	DOB:
Vasquez	Lucia	1953-09-10
Address:	Phone:	
4041 44Th St
San Diego, California 921051603	6198237353	

Sex:

☐ Male ☐ Female ☐ Other

Date Requested: **Member ID:**

Requesting Provider and Specialty:

Clinical Indications for the Services:

REQUESTED SERVICES

CPT/HCPC Codes(s)	Description	Number of Units

Follow up appointment:

Patient has scheduled appointment for date at : am/pm for minutes

Location of appointment:

If requesting more codes, use another form and number pages.

If scheduling an appointment, please allow 72 hours for request to be reviewed.

****Required: Attach clinical documentation to this form upon submission****

Fax number to return approved authorization:



FAMILY HEALTH CENTERS OF SAN DIEGO
823 GATEWAY CENTER WAY
SAN DIEGO, CA 92102

Face Sheet

Printed on: 02/03/2026

Patient: VASQUEZ, LUCIA MR# 1364779 MasterId: 101386778
DOB: 09/10/1953 Age: 72 Yrs 4 Mos 24 Days Gender: FEMALE Marital Status: DIVORCED
Address: 4041 44TH ST EMail: lucyvasquez53@gmail.com
SAN DIEGO CA 92105
Contact Phone No:
HOME: (619)964-6523
MOBILE: (619)964-6523
ALTERNATE: (619)823-7353

Ethnicity: MEXICAN, MEXICAN Education: GRADE 8 Language: SPANISH
Race: AMERICAN, CHICANO/A Family Size: 0
DON'T KNOW

Home Facility: Home Dept:
First Enc: 18938921 First DOS: 10/08/2024
Facility: PACE Dept: PACE ENROLLMENT SF Disc.: 0%

Patient Funding Resource

Payer	Policy #	Valid From	Valid To	Primary Provider
PACE Family Health Center	101386778	11/01/2024	12/31/2026	PROVIDER, NOT SELECTED
SELF PAY		10/09/1953		PROVIDER, NOT SELECTED

Most Recent Encounter:

EncounterId: 22783316 DOS: 01/28/2026 Payer: PACE Family Health Center
Facility: PACE Dept: PACE REHAB Provider: MORENO, TOM

Contact:

Contact Person:	Program	Start Date	EndDate
RAMON VAZQUEZ	Enroll CG	11/01/2024	
Contact Type: CHILD	PACE CG	11/01/2024	
Contact Address: CA	C19Vaccine	11/11/2023	
Home Phone:	PR All	01/19/2026	02/04/2026
Work Phone:	PR All	01/26/2026	02/11/2026
	PR All	02/02/2026	02/18/2026
	TeleAppt	01/19/2026	02/04/2026
	TeleAppt	01/26/2026	02/11/2026
	TeleAppt	02/02/2026	02/18/2026
	Hedis	10/23/2024	

Use for Prescription Delivery or Mail Order Services - 340B Program

☐ Community Medical Center Pharmacy ☐ Hillcrest Pharmacy

Service Type: ☐ Delivery ☐ Mail Order

Note: Do not release Problem List without patient consent.

This report is for the sole use of the intended recipient(s) and may contain confidential and privileged information, including Protected Health Information. Any unauthorized review, use, disclosure or distribution is prohibited. If you are not the intended recipient, please contact the sender by e-mail and destroy all copies of the report.

Delivery Time: ☐ AM before Noon Next Day ☐ PM after Noon Next Day

(Excludes Hillcrest Pharmacy)

☐ *Must Deliver Today AM before Noon ☐ *Must Deliver Today PM after Noon

**Note: Same day delivery is limited to new medications, antibiotics, or urgent cases requested by the provider (if faxed by 5pm) and pain medications (if faxed by 3:30pm).*

Address for the Service Type: ☐ Same as Above ☐ Other (list address below)

Phone Number for Payment by Credit/Debit Card or ☐ Same as Above ☐ Other: _____
Insurance Information:

Meds on Time Service (Hillcrest Pharmacy Only) ☐ Yes

Fax all prescriptions plus Face Sheet to Community Medical Center Pharmacy at 619-263-1557.

Fax all prescriptions plus Face Sheet to Hillcrest Pharmacy at 619-260-1031.

Note: Do not release Problem List without patient consent.

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Active Medication List as of 2/3/2026

Family Health Centers of San DiegoPatient Name: VASQUEZ, LUCIA 09/10/1953

Drug Name	Drug Information	SIG	StartDate	Qty	Refil
Aspirin Low Dose [Aspirin]	81 mg (oral)	Take orally 1 daily New med per Cardiology, please send next cycle (labels in Spanish) please SEND	03/04/2025	90	3
Atorvastatin Calcium [Atorvastatin Calcium]	20 mg (oral)	Take orally 1 at bedtime to reduce cholesterol and risk of heart attacks and strokes SEND next cycle	10/29/2025	60	6
Gabapentin [Gabapentin]	300 mg (oral)	Take orally 1 at bedtime next cycle send	10/16/2025	30	3
hydrALAZINE HCl [Hydralazine HCl]	25 mg (oral)	Take orally 1 twice daily SEND next cycle	10/29/2025	60	3
Ketoconazole [Ketoconazole (Topical)]	2 % (External)	Apply twice daily for 30 Days	01/20/2026	60 g	6
Konsyl Daily Fiber [Psyllium]	100 % (oral)	Take orally 1 tbsp daily (As Needed) Use as directed as needed for constipation, labels in Spanish, please --> SEND	11/19/2024	1 bottle	3
Lasix [Furosemide]	20 mg (oral)	Take orally 1 daily Labels in spanish please. SEND next cycle	10/29/2025	30	6
Losartan Potassium [Losartan Potassium]	50 mg (oral)	Take orally 1 twice daily Labels in SPANISH, cycle refill	12/10/2025	30	5
Melatonin [Melatonin]	10 mg (oral)	Take orally 1 at bedtime Please send STAT and labels in Spanish	10/29/2025	30	11
Simbrinza [Brinzolamide- Brimonidine Tartrate]	1-0.2 % (ophthalmic)	Apply 1 every morning 1 drop bilateral eyes every morning. SEND STAT, labels in Spanish	10/07/2025	8 ml	6
Timolol Maleate [Timolol Maleate (Ophth)]	0.5 % (ophthalmic)	Apply 1 gtt OU every morning Send STAT labels in Spanish	10/07/2025	10 mL	6
Vitamin D [Ergocalciferol]	50 MCG (2000 UT) (oral)	Take orally 1 daily for 30 Days	11/05/2024	30	0
Xalatan [Latanoprost]	0.005 % (ophthalmic)	Apply 1 gtt OU every evening 1 drop OU. SEND NEXT CYCLE LABELS IN SPANISH	12/04/2025	1 bottle	3



22021461-E

MasterId: 101386778

Phone: (619)823-7353

VASQUEZ, LUCIA**MR#: 1364779****DOB: 09/10/1953** 72YR 1MO F

PACE/ADULTS

PSR: SILVIAROB

P: PACE FHC[101386778]

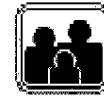
ENC: 22021461

Prov: BUGARIN, MAYRA AILED

PGM: Hedis

Printed: 02/03/2026 11:16 AM

DOS: 10/29/2025

**FAMILY HEALTH CENTERS OF SAN DIEGO****Allergies:**

NKDA

Chief Complaint:

Follow-up: Annual assessment

Vital Signs:

BP		T	P	R	WT		HT		BMI		BW	HC
136	84	0.0	85	0			0.00"		0.00			0.00
145	76	98.1	94	20	100lbs 14oz 45.76 Kgs		59.50"		20.00			0.00

Other Data/Comments: , O2:97%

Subjective:**CC:** Annual Assessment

HPI: Mrs. Vasquez, Lucia is a 71 yo, former production inspector, bilingual, with PMHx significant for Charcot-Marie-Tooth disease (diagnosed at 42 yo), hereditary motor and sensory neuropathy, essential HTN, on BiPAP (at night) in the setting of acute on chronic hypercapnic and hypoxemic respiratory failure secondary to restrictive lung disease due to chronic neuromuscular weakness, presenting to PACE clinic for CC as above.

Interval Events:

- 05/21/25: Last Semi Annual Assessment
- No falls
- No ER visits
- No hospitalizations

PMHx, PSHx, Family Medical Hx:

No changes, please see note from 11/05/25 for further details

The following is based on the 4Ms framework for providing age-friendly care to older adults:

Mentation: Alert and oriented x4, Mini-Cog performed on 05/21/25 with a total score of 5/5, able to recall 3 words and normal clock draw.

Mobility: She uses a 4 wheel walker, she can walk short distances such as bed to restroom, and she remains mainly at home, she spends more of her day sitting. She has also a cane. She wears compression socks and AFO's for her bilateral foot drop

Medications: Medication reconciliation completed today per Rx Tab. Please see below in meds.

What Matters the Most: to continue to be independent as much as she can for the time that she can, complete goals of care and to remain at home avoiding hospitalizations, to continue her medical care following regularly with us and specialty care.

ADLs***

Patient is independent in bathing

Patient is independent in dressing

Patient is independent in toileting

Patient is independent in transferring

Patient is independent in maintaining continence

Patient is independent in feeding

IADLs***

Patient needs assistance with telephone

Patient needs assistance with shopping

Patient needs assistance with housework

Patient needs assistance with laundry

Patient needs assistance with preparing meals

Patient does not drive

Patient is independent in taking medications

Patient is independent in managing money

FALLS***

Denies falling since October 2023

Denies falling >2 times in the past year

States sometimes fears of falling due to feeling unbalance and her legs are weak when she changes positions or when she is not using DME

URINARY INCONTINENCE***

Positive for losing urine (urinary incontinence)

VISUAL IMPAIRMENT***

Eyesight (uses prescription glasses to read) does not interfere with patient's watching TV, reading, or regular daily activities.

Right eye has half of her vision (had laser 2022), uses prescription glasses given by PACE

Glaucoma, per Opt on 08/04/25: IOP stable, pt to continue with all drops as instructed and continue with Dr. Shaw (f/u 3 months for IOP check)

Pseudophakia: clear and centered, monitor.

Senile Cataract, Epiretinal membrane: Pt to keep appt with Dr. Najafi

HEARING IMPAIRMENT***

Patient having trouble to listen. She was seen by ENT/Audiology recommended hearing aids for SNHL. Patient now agrees to them.

01/24/25. 02/18/25 ENT recs reviewed, follow-up in 1 year.

DEPRESSION***

PHQ-9 score 2

Denies little interest or pleasure in doing things feeling down, depressed, or hopeless

Denies SI or HI

OCCUPATION/SOCIAL SUPPORT***

Social Hx: Mother of 2, divorced (2006), was born in Guadalajara, and started living in the US in 1985. Used to work in a company that builds micro-chips for the military. Used to be a production inspector, she retired at 65 yo. Has 2 kids, 43 yo and 35 yo, they lived in SD area. Lives with daughter in-law Mirna and her son, she is paying her own house.

Tobacco: none

Etoh: none

Illicit Drugs: none

MEDICATIONS* Med rec 10/29/25**

- Aspirin 81 mg po daily
- Atorvastatin 20 mg po QHS
- Gabapentin 300 mg po QHS
- Hydralazine 25 mg po BID
- Ketoconazole 20 mg/ml topical apply BID

- Psyllium 6000 mg powder for oral suspension, 1 tbsp daily
- Furosemide 20 mg po daily
- Losartan 50 mg po BID
- Melatonin 10 mg po QHS
- Brimonidine tartrate 2mg/ml/Brinzolamide 10 mg/ml apply 1 gtt OU every morning
- Timolol 5 mg/ml, 1 gtt OU every morning
- Ergocalciferol 0.05 mg po 1 daily
- Latanoprost 0.05 mg/ml, apply 1 gtt OU every evening

OTC drugs:

Once in a while omeprazole and naproxen

ALLERGIES/ADR: NKDA**HEALTH CARE MAINTENANCE**

Breast cancer screening: Screening mammogram completed on 12/20/24, BI-RADS assessment category 1-negative

Cervical cancer screening: No records from last pap smear, outside FHCSO, per pt was told normal.

Colon cancer screening: Referral completed needs clearance from pulm and cards.

Osteoporosis screening: Needs specific accommodation with bipap to be able to complete procedure.

Immunizations: Due for influenza, accepted to take it today.

OTHER MEDICAL PROBLEMS:

GLAUCOMA, advanced, by discs. IOP acceptable on drops, but likely too high for health of nerves.

MACULAR DISEASE OU.

Seen by Ophthalmology on 05/16/25 and then 08/18/25 within FHCSO. Has follow-up on 11/21/25

States she is using all drops.

ONYCHOMYCOSIS**EDEMA OF LOWER EXTREMITY**

Seen by Podiatry on 10/14/25 within FHCSO. Patient state following all recs per podiatry and has follow-up in 3 months.

CHARCOAT-MARIE-TOOTH-DISEASE**HEREDITARY MOTOR AND SENSORY NEUROPATHY**

Chronic, hemodynamically stable in no acute resp distress, O2 sat 97% on RA.

Patient has been evaluated by Neuromuscular consultation on 12/02/24 at UCSD by Dr. Williams Allen Ueng. Recs appreciated.

The following was ordered by Neuromusc UCSD:

- EMG/NCS with Dr. Lavery UCSD
- Pulmonary function test UCSD ----> completed on 01/03/25 at UCSD
- Referral to pulmonology Dr. Orr or Dr. Chen
- Blood work (a1c, B12, SPEP with immunofixation, Invitae Comprehensive Neuropathies panel) - labs were completed at UCSD
- PT referral - Follows at PACE
- Follow-up with Dr. Ueng William Allen

NEUROMUSCULAR RESPIRATORY WEAKNESS ON BIPAP, CHRONIC

RESTRICTIVE LUNG DISORDER IN PFT'S

HYPERCAPNIC, ACUTE ON CHRONIC RESPIRATORY FAILURE

- Pulmonary records reviewed from Dr. Orr on 04/17/25, recs appreciated. Therefore will need to get data from her device and send it to UCSD pulmonary for likely make adjustments.
- She also has dysphonia. Vocal cord paralysis is also common in CMT. She does not report prior intubation. Referral to ENT to confirm the issue and see if any therapeutics can be offered.
- Asymmetric LE edema. Notes longstanding left leg greater than right swelling, as well as weakness. She does follow with a cardiologist for a history of heart failure.
- Importance of consistent use of NIV even when hospitalized or getting procedures. Discussed importance of self advocacy.

OLD MYOCARDIAL INFARCTION

HX OF NSTEMI 2021

Chronic, asymptomatic. Follow-up with Cardiology completed on 03/04/25 and 09/04/25 at UCSD records reviewed. On daily aspirin and to complete Nuc Med Myocardial perfusion Stress and ECHO.

- Continue aspirin 81 mg po daily
- Referrals for Nuc Med Myocardial perfusion Stress completed on 05/30/25:

IMPRESSION:

1. Normal left ventricular myocardial perfusion study. No evidence for pharmacologically inducible ischemia.
2. No evidence for prior transmural myocardial infarctions.
3. Normal LV chamber size. Normal left ventricular global function. Estimated left ventricular ejection fraction

is 70%

- Needs TTE for mildly RVSP. - 6-9 months, follow-up in 9 months
- Will ask referrals for an update

HYPERLIPIDEMIA

Chronic, at goal. Family history positive for high cholesterol, mother died from stroke. Total cholesterol high with high HDL. High ASCVD 10-year risk >7.5%. Moderate to high intensity statin rec. Risk vs benefits discussed during last appt same as side effects plus healthy lifestyle interventions. Today states no side effects.

- Continue Atorvastatin 20 mg po QHS

INSOMNIA

Chronic, now stable. Hard to fall asleep, once asleep able sleep for couple of hours. Falling asleep until 3 am watching TV. States worries a lot.

- Sleep hygiene discussed and counseling in techniques to approach a better sleep.
- On melatonin 10 mg at bedtime, has been helping more
- Continue MH once a month, on therapy states she likes it and would like to continue

MYOPIA

REGULAR ASTIGMATISM

PRESBYOPIA

- Saw optometry has prescription glasses.

ESSENTIAL HYPERTENSION

Chronic, at goal <140/90. Continue with HTN regimen which includes:

- Continue Losartan 50 mg po BID
- Continue Hydralazine 25 mg po BID

SNHL

Saw ENT on 02/18/25 due to hearing loss. Audiogram completed with evidence of SNHL, patient deferred hearing aids at that time, today she wants them as is getting worse. Per records from ENT on 02/03/25 SNHL audiogram obtained with results as above and discussed with the patient. No significant asymmetry on audiogram or significant change compared to her 2021 audiogram to suggest retrocochlear pathology as cause of symptoms, however due to persistent vertigo episodes with unilateral tinnitus symptoms will obtain MRI IAC w/o contrast to r/o middle ear pathology as cause of symptoms. Follow-up in 6 months.

- At this point will refer back to them to complete hearing aids as patient is now interested in them.

SCREENING FOR COLON CA

GI appt on 01/31/25, Records reviewed. Golytely ok not to be send for now. No FHx of colon cancer. Fit test completed on 11/21/24 and negative. Denies blood in stool, diarrhea or constipation. Has BM regularly. Will

need to schedule once pt is cardio/pulm cleared.

ROS:

General: Fever **no** . Chills **no** . Headache **no** . Bodyache **no**

ENT: Ear pain **no** . Ear discharge **no** . Nasal discharge **no** . Sore throat **no** . Difficulty swallowing **no**

Eyes: eye pain **no** eye discharge **no**

Resp: Cough **no** . Shortness of breath **no** , Wheezing **no**

Cardio: Chest pain **no** . Palpitations **no**

GI: Nausea **no** . Vomiting **no** . Diarrhea **no** . Constipation **no** . Abdominal pain **no**

GU: Dysuria **no** . Frequency **no** . Hematuria **no** . Polyuria **no** .

Skin: Rash **no** . Itching **no** .

Additional symptoms: None

Objective:

BP: 145/76 **HR:** 94 **RR:** 20 **T:** 98.1 **WT:** 100 lbs. 14 oz. **BMI:** 20.03 **O2 sat:** 97%

BP: 136/84 **HR:** 85

Vitals reviewed

GEN: alert, awake. NAD.

HEENT: MMM, NC/AT, oropharynx clear, missing teeth, cavities observed, PERRLA. EOMI b/l. TM's clear.

NECK: no JVD. no TM. no carotid bruit b/l. supple.

CV: RRR. no M/R/C noted.

CHEST: diminished breath sounds on auscultation b/l more at bases than apex. no W/R noted.

ABD: positive BS. NT/ND. no HSM.

EXT: no clubbing or cyanosis, positive 1+ edema bilaterally R>L, positive thickening brittle toenails, Pulses + UE/LE b/l. She wears compression socks and AFO's for her bilateral foot drop

SKIN: moist, no loss of hair

NEURO: alert and oriented x 3. Cranial nerve exam 2 through 12 intact. Motor 4/5 in flexibility. Sensory intact to light touch

Assessment/Plan:

PACE On-Going Medical Evaluation- V700[185175005].

Mrs. Vasquez, Lucia is a 71 yo, former production inspector, bilingual, with PMHx significant for Charcot-Marie-Tooth disease (diagnosed at 42 yo by Neurologist here in SD), hereditary motor and sensory neuropathy, essential HTN, on BiPAP (at night) in the setting of acute on chronic hypercapnic and hypoxemic respiratory failure secondary to restrictive lung disease due to chronic neuromuscular weakness, presenting to PACE clinic for Annual Assessment

Thrombocytopenia- 2875[I10: D69.6]

Mild 126.00 on 05/21/25. previous PLT back in Nov 2024 were normal, no signs of active bleeding, will repeat labs, new med aspirin prescribed by Cards around March 2025. Labs negative including HIV, HepC, B12, folate .

- Labs schedule for Jan per pt request

At risk for falls- V1588[I10: Z91.81].

No falls in the last 12 moths.

- Continue fall prevention class
- Continue PT PACE prn
- Continue FWW
- Continue compression socks and AFO's for her bilateral foot drop

Depression screening- V790[I10: Z13.31].

PHQ-9 total score 2, no SI or HI.

- Continue mental health therapy once a month, pt likes it and shows compliance.

Old myocardial infarction- 412[I10: I25.2]

History of non-ST elevation myocardial infarction (NSTEMI) 2021.

Chronic, asymptomatic. Follow-up with Cardiology UCSD, recs to start daily aspirin and to complete Nuc Med Myocardial perfusion Stress (completed on 05/30/25 normal)

- Continue aspirin 81 mg po daily
- Complete TTE for mildly RVSP 9 months
- Follow-up in 9 months
- Referral ordered

Essential hypertension 4019[I10: I10].

Chronic, at goal <140/90. Continue with HTN regimen which includes:

- Continue Losartan 50 mg po BID
- Continue Hydralazine 25 mg po BID

Charcot-Marie-Tooth disease- 3561[I10: G60.0]

Diagnosed at the age of 42 yo by neurology .

Hereditary motor and sensory neuropathy- 3562[I10: G60.0].

Chronic, hemodynamically stable in no acute resp distress, O2 sat 97% on RA.

Patient has been evaluated by Neuromuscular consultation on 12/02/24 at UCSD by Dr. Williams Allen Ueng.

Recs appreciated.

Genetically confirmed TRPV4 mutation, CMT 2C; axonal polyneuropathy (OMIM #606071); autosomal dominant; confirmed 12/2024

-onset in childhood with difficulty walking, inability to run, and jump

-unable to open bottles in her 40's

-diagnosed with CMT after having an EMG at age 46y

-started using cane for ambulation in her mid 40's

-started using a walker in latter half of 2024

-hospitalized from 6/23/24 to 7/3/24 due to respiratory failure, possibly due to CMT

Her EMG/NCS completed 7/18/25 showed a length dependent sensorimotor axonal polyneuropathy, consistent with the CMT 2C diagnosis.

01/2025 PFT is consistent with severe neuromuscular respiratory weakness (restrictive pattern). She recently established with Dr. Orr and continues on nightly BiPAP.

Continue to follow with pulmonology (Dr. Orr) for nocturnal bipap

Continue physical therapy

Compression stockings for LE edema

Recommend she wear AFOs for her bilateral foot drop

Patient's specialty appt need to be reschedule (referrals notified) pt states "no one told her"

Sleep Apnea- 78057[I10: G47.30] on BiPAP.

Breathing-related sleep disorder- 78057[I10: G47.39].

Please see note from 10/07/25

Neuromuscular respiratory weakness on Bipap

Restrictive lung disease

Hypercapnic, acute on chronic respiratory failure

Chronic, compliance with bipap at night.

- Pulmonary records reviewed from Dr. Orr on 08/13/25, recs appreciated:

Impacts respiratory muscles and diaphragm; lung sounds are clear, but diaphragm movement is limited. A lung function test is planned before the next neurology appointment to monitor respiratory status. There are planned ventilator setting adjustments to ensure adequate breathing support, alleviate heart stress, improve sleep quality, and enhance well-being. There have been no recent hospitalizations or respiratory infections.

The patient uses a Trilogy Evo ventilator nightly for 8 hours. Initially, they feel claustrophobic but adjust and fall asleep. The prior CO2 level was 62 but current TcCO2 is better yet slightly elevated still, and low minimum

pressure settings were noted. We will perform ventilator setting adjustments to optimize breathing support. The patient does not use daytime oxygen but uses 2 L of oxygen at night. The patient was advised to consult an ENT specialist if voice issues persist, as they may be related to CMT affecting the laryngeal nerves. There has been no recent ENT consultation, potentially due to insurance approval issues. There was a discussion on possible treatments and recommendations by the ENT specialist.

Follow-up: The next neurology clinic visit is scheduled for next month, with a follow-up in several months.

Return to clinic: Return in about 6 months (around 2/13/2026).

Glaucoma- 3659[I10: H40.9].

Disorder of macula of retina- 36289[I10: H35.9].

Impairment Level: Blindness, One Eye - Low Vision Other Eye36910[I10: H54.10]

Impaired vision of the right eye .

Myopia- 3671[I10: H52.13].

Regular astigmatism- 36721[I10: H52.229].

Presbyopia- 3674[I10: H52.4].

Epiretinal membrane- 6289[I10: H35.9].

Pseudophakia- 37939[I10: Z96.1].

Senile cataract- 36610[I10: H25.9].

Glaucoma advanced, by discs. IOP acceptable on drops, but likely too high for health of nerves.

Macular disease OU.

Seen by Ophthalmology on 05/16/25 and then 08/18/25 within FHCSO.

- Has follow-up on 11/21/25

- Continue using all drops.

Insomnia-78052[I10: G47.00].

Chronic, now stable. Hard to fall asleep, once asleep able sleep for couple of hours. Falling asleep until 3 am watching TV. States worries a lot.

- Sleep hygiene discussed and counseling in techniques to approach a better sleep.

- On melatonin 10 mg at bedtime, has been helping more

- Continue MH once a month, on therapy states she likes it and would like to continue

Dyslipidemia- 2722[I10: E78.5].

Chronic, at goal. Family history positive for high cholesterol, mother died from stroke. Total cholesterol high with high HDL. High ASCVD 10-year risk >7.5%. Moderate to high intensity statin rec. Risk vs benefits discussed during last appt same as side effects plus healthy lifestyle interventions. Today states no side effects.

- Continue Atorvastatin 20 mg po QHS

Sensorineural hearing loss, bilateral- 38918[I10: H90.3].

Saw ENT on 02/18/25 due to hearing loss. Audiogram completed with evidence of SNHL, patient deferred hearing aids at that time, today she wants them as is getting worse. Per records from ENT on 02/03/25 SNHL audiogram obtained with results as above and discussed with the patient. No significant asymmetry on audiogram or significant change compared to her 2021 audiogram to suggest retrocochlear pathology as cause of symptoms, however due to persistent vertigo episodes with unilateral tinnitus symptoms will obtain MRI IAC w/o contrast to r/o middle ear pathology as cause of symptoms.

- At this point will refer back to them to complete hearing aids as patient is now interested in them.

Onychomycosis- 1101[I10: B35.1].

Chronic, stable.

Seen by Podiatry

- Debrided nails x10 without incidence
- Continue ketoconazole 2%
- Recs to elevate b/l foot with 1 pillow at night
- RTC for f/u in 3 months
- Patient to continue using compression socks

Health Care Maintenance

Breast cancer screening: Screening mammogram completed on 12/20/24, BI-RADS assessment category 1-negative, repeat in 1-2 years.

Cervical cancer screening: No records from last pap smear, outside FHCSD, per pt was told normal. defers last screening.

Colon cancer screening: Referral completed needs clearance from pulm and cards.

Osteoporosis screening: Needs specific accommodation with bipap to be able to complete procedure.

Immunizations: Influenza updated

Transitions of care

Patient aware about PCP Dr. Bugarin's last day in clinic.

- Follow-up with new PCP in 3 months

Mayra A. Bugarin, MD

Geriatric Medicine

- Per pt requesting to do labs not until January 2026 (orders in)

- Asking for supplies for Bipap machine every 3 months (needs them in late November)

The following appts need to be reschedule pt states "no one let her know about this"

Patient using BiPAP needs supplies every 3 months please approve.

Current DME: Kadence Healthcare

Brand: Phillips Respironics Trilogy Evo

Type: Noninvasive Positive Pressure Ventilation

Device Usage: Night time

Night time Interface: Full Face

Nocturnal Mode & Settings: AVAPS-AE

PS min/max: 3-10 cm H₂O

EPAP: 4-5 cm H₂O

Max Pressure: 14 cm H₂O

VT: 235 mL

Breath Rate: 12 bpm

Trigger type: Auto-trak

Rise 2

AVAPS Speed: 3 cm H₂O/sec

Ti: 1.2 sec

Finalized By:

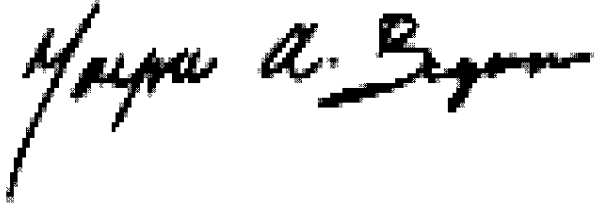


MAYRA BUGARIN on 11/05/2025

Addendum

• **DME** : OTHER-6 : REFERRAL SPECIALIST TO SELECT DME
(VERIFY AUTH REQUIREMENT)

Finalized By:



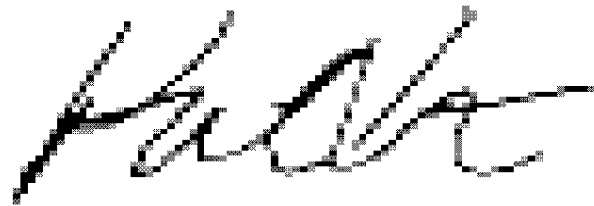
MAYRA BUGARIN on 11/05/2025

pharmacy requesting medication refill:

• **Losartan Potassium** : [losartan potassium 50 MG Oral Tablet] ▶ **06/18/2025** Qty : **30** R : **5** (Take orally 1 twice daily
Labels in SPANISH, SEND next cycle

 Grane Supply - Denver 3251 Revere Street Aurora, CO 800111847)

Finalized By:



PATRICIA CASTILLO on 12/10/2025

1x Order for Escort 01/12/25 7:30am-8:45am

1. CNA, Spanish speaking, to provide escort for medical appt

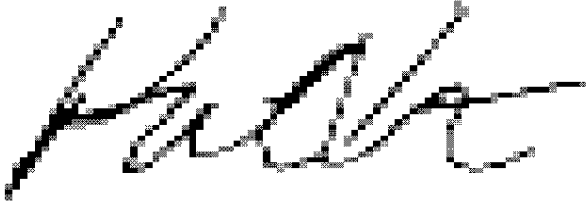
2. Please meet participant at ~7:30am for 8am pulmonology/neurology multidisciplinary appointment: 4510 Executive Drive, Ste. 325,

Patient Name: **VASQUEZ, LUCIA** DOB: 09/10/1953 MR#: 1364779

San Diego, CA 92121

3. Assist as needed and remain with ppt until safely picked up by PACE transportation

Finalized By:



PATRICIA CASTILLO on 01/09/2026

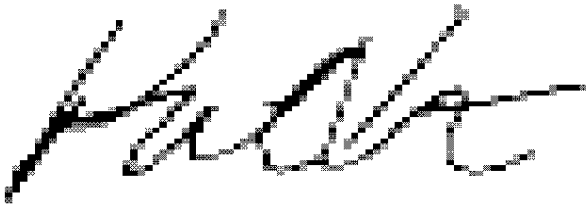
1x Order for Escort 01/20/2026 1pm-3pm

1. CNA, Spanish speaking, to provide escort to medical appt
2. Please meet participant at 1pm for 130pm appt at FHCC Podiatry

1550 BROADWAY ste 2 San Diego, CA 92101

3. Assist as needed and remain with participant until safely picked up by PACE transportation

Finalized By:



PATRICIA CASTILLO on 01/19/2026

1x Order for Escort 02/04/2026 12:30pm-2:30pm

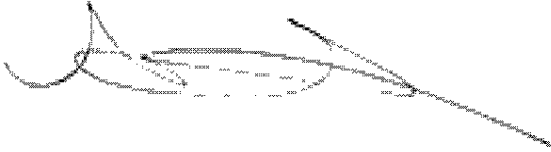
1. CNA, Spanish Speaking, to serve as escort for medical appt
2. Please meet participant at 12:30pm at 12:45pm at Lab

4520 Executive Dr. Ste T2 San Diego, CA 92037

3. Assist as needed and remain with participant until safely picked up by PACE transportation

• **Consult** : HOME CARE (PACE ONLY) (VERIFY AUTH REQUIREMENT) [Referral Valid until 08/07/2026] Consult,

Finalized By:



RENDA DAWUD on 02/03/2026

Active Problem List (Diagnosis)

Problem List

Anemia screening
 At risk for falls
 Breathing-related sleep disorder
 Charcot-Marie-Tooth disease
 Diabetes mellitus screening
 Dyslipidemia
 Edema of lower extremity
 Essential hypertension
 Insomnia
 Lipid screening
 PACE Initial Medical Evaluation
 Screening for depression
 Screening for malignant neoplasm of colon
 Screening for metabolic disorder
 Thyroid disease monitoring status

Acute Diagnoses

Z130	Anemia screening
Labs	CBC w/ AutoDiff (Sysmex) - Fasting: No, Order Date: 01/07/2026 Draw Date: N/A
Z9181	At risk for falls
Consult	Home Care (Pace Only) , Services Ordered: Consult, Other Service(s): [CARE BRIGHTSTAR] Reason:1x Order for Escort 01/20/2026 1pm-3pm 1. CNA, Spanish speaking, to provide escort to medical appt 2. Please meet participant at 1pm for 130pm appt at FHCC Podiatry 1550 BROADWAY ste 2 San Diego, CA 92101 3. Assist as needed and remain with participant until safely picked up by PACE transportation , STAT

Consult	Home Care (Pace Only) , Services Ordered: Consult, Other Service(s): [CARE BRIGHTSTAR] Reason:1x Order for Escort 02/04/2026 12:30pm-2:30pm 1. CNA, Spanish Speaking, to serve as escort for medical appt 2. Please meet participant at 12:30pm at 12:45pm at Lab 4520 Executive Dr. Ste T2 San Diego, CA 92037 3. Assist as needed and remain with participant until safely picked up by PACE transportation , STAT
Z131	Diabetes mellitus screening
Labs	Hemoglobin A1C - Fasting: No, Order Date: 01/07/2026 Draw Date: N/A
Z13220	Lipid screening
Labs	LIPID PANEL w/ DIRECT LDL - Fasting: No, Order Date: 01/07/2026 Draw Date: N/A
Z0001	PACE Initial Medical Evaluation
Medication	HydrALAZINE Hydrochloride - 25 mg oral (Allow Substitution) Instruction:SEND next cycle SIG:Take orally 1 twice daily SEND next cycle
Medication	Losartan Potassium - 50 mg oral (Allow Substitution) Instruction:Labels in SPANISH, cycle refill SIG:Take orally 1 twice daily Labels in SPANISH, cycle refill
Z1211	Screening for malignant neoplasm of colon
Labs	OCCULT BLOOD, STOOL, IMMUNOCHEMICAL (Polymedco) - Fasting: No, Order Date: 10/29/2025, Draw Location: PACE COLLEGE GROVE Draw Date: 12/01/2025
Z1329	Screening for metabolic disorder
Labs	COMP. METABOLIC PANEL (17) w/ GFR - Fasting: No, Order Date: 01/07/2026, Draw Location: PACE COLLEGE GROVE Draw Date: N/A
Labs	THYROID-STIMULATING HORMONE W/REFLEX TO FREE T4 - Fasting: No, Order Date: 01/07/2026 Draw Date: N/A
Z1211[275978004]	Nursing
Nursing	Log Dispensed Or Administered Medication [Completed] Instruction:FIT Kit. Note: if this is a telehealth visit then set order status to FUTURE. It will be mailed from a central location
Z1331[1120010001]	Nursing
Nursing	Phq9 [Completed]
Nursing	Gad7 [Completed]
Chronic Diagnoses	
G600	Charcot-Marie-Tooth disease
Consult	Home Care (Pace Only) , Services Ordered: Consult, Other Service(s): [CARE BRIGHTSTAR] Reason:1x Order for Escort 01/12/25 7:30am-8:45am 1. CNA, Spanish speaking, to provide escort for medical appt 2. Please meet participant at ~7:30am for 8am cardiology appointment: 4510 Executive Drive, Ste. 325, San Diego, CA 92121 3. Assist as needed and remain with ppt until safely picked up by PACE transportation, STAT
E785	Dyslipidemia
Medication	Atorvastatin Calcium - 20 mg oral (Allow Substitution) Instruction:to reduce cholesterol and risk of heart attacks and strokes SEND next cycle SIG:Take orally 1 at bedtime to reduce cholesterol and risk of heart attacks and strokes SEND next cycle

R600	Edema of lower extremity
Medication	Lasix - 20 mg oral (Allow Substitution) Instruction:Labels in spanish please. SEND next cycle SIG:Take orally 1 daily Labels in spanish please. SEND next cycle
G4700	Insomnia
Medication	Melatonin - 10 mg oral (Allow Substitution) Instruction:Please send STAT and labels in Spanish SIG:Take orally 1 at bedtime Please send STAT and labels in Spanish
I10[59621000]	Nursing
Nursing	Vitals [Completed] Instruction:Manual BP recheck in 5 minutes. Make sure patient's feet are on the floor and upper arm is at level of heart. Patient should be relaxed.

Other Orders

No Diagnosis Code	Nursing
Nursing	Chief Complaint [Completed]
Nursing	Allergy [Completed]
Nursing	Vitals [Completed]
Nursing	O2 Saturation/Peak Flow [Completed]
Patient Education	Colorectal Cancer Screening and Testing Guidelines - Spa
Follow Up	Follow up in for Follow-up in 3 months new PCP
Template	Core Screening
Template	Social Determinants Of Health
Template	Smart Goal
Template	Health Education
Template	Dexa Manage Order
Template	Cc Fit Due Soon
Template	Bp High Recheck
Template	Immunizations

Electronically signed by:



Date: 11/03/2025

(MAYRA BUGARIN [MD] License: A183933 NPI:
1790313922)

Co Signature:



Date: 01/19/2026

(PATRICIA CASTILLO [PA] License: 17220 NPI:
1376550657)